Barcode No	**89781298**	Lab No	16952502200003
Patient Name	**Mrs.SUNIT KOUR**	Reg Date	20/Feb/2025 03:16PM
Age/Sex	32 YRS/Female	Sample Coll. Date	20/Feb/2025 02:20 PM
Refered By	DR. GOVT HOSPITAL GANDHINAGAR	Sample Rec.Date	21/Feb/2025 08:10 AM
		Report Date	21/Feb/2025 10:45AM

HAEMATOLOGY

Test Name With Methodology	Result	Unit	Biological Ref.Interval
Haemoglobin HPLC			
Hb F(Foetal Hb) Level	1.8	%	< 2.0
Peak 2	5.1	%	
Peak 3	2.6	%	<10
Hb A0	81.8	%	
HbA2	2.8	%	1.9 - 3.5
Hb D (Punjab)	0.0	%	<0.02
Hb Sickle	0.0	%	<0.02
Hb C	0.0	%	<0.02
Hb E	0.0	%	<0.02
Opinion	Normal Pattern		
Advise	Please Correlate Clinically		

Comment:

Haemoglobinopathy is a kind of genetic defect that results in abnormal structure of one of the globin chains of the hemoglobin molecule. Hemoglobin variants are abnormal forms of hemoglobin. All results have to be correlated with age and history of blood transfusion. If there is history of blood transfusion in last 3 months, repeat testing after 3 months from last date of transfusion is recommended. ***In case of hemoglobinopathy, parents or family studies, DNA Analysis and Genetic counseling is advised.***

~ **P3 window**- Above 10% is often indicative of either denatured forms of hemoglobin (degenerated sample) or may suggest a possibility of abnormal hemoglobin variant. Hence, repeat analysis with fresh sample or DNA studies is advised.
~ **P2 window**- Above 10% is indicative of either glycated hemoglobin requiring correlation with diabetic status or may suggest a possibility of abnormal hemoglobin variant requiring further DNA studies for confirmation.
~ Screening of the spouse are strongly recommended, if pregnant woman have hemoglobinopathy.
~ This test detects Beta thalassemia and hemoglobinopathies. DNA analysis is recommended to rule out alpha thalassemia and silent carriers.
~ Mild to moderate increase in **fetal hemoglobin (HbF)** can be seen in some acquired conditions like Pregnancy, Megaloblastic anemia, Thyrotoxicosis, Hypoxic conditions, Chronic kidney disease, Recovering marrow, MDS, Aplastic anemia, PNH, Medications (Hydroxyurea, Erythropoietin, Butyrates) etc.
~ Iron deficiency Anemia may be associated with **low HbA2** result and may mask thalassemia condition also.
~ **Elevated levels of HbA2** are seen in Beta-Thalassemia, and also in other conditions like Sickle cell Alpha-Thalassemia, Megaloblastic Anemia, Unstable Hemoglobinopathies, Zidovudine- treated HIV patients, Congenital Dyserythropoietic Anemia (CDA) type 1 and Hyperthyroidism.
***Results relate only to the sample, as received.**

Prashant.
Dr. Prashant Goyal *(DCP)*
(Director & Chief Pathologist)
Reg. No. DMC-53016

Chromatogram Report

Tosoh HLC723G11 V3.07 1 2025-02-21 10:28:15

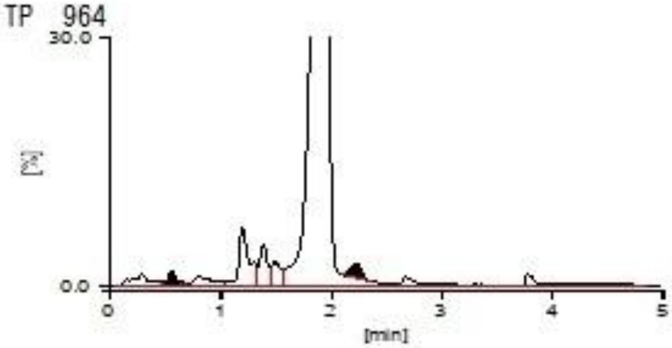
ID 89781298
Sample No. 2025022110060052 SL 0004 - 08
Patient ID
Name
Comment

CALIB F Y =1.3028X + 0.3803
A2 Y =1.4105X + 0.7960

Name	%	Time	Area
F	1.8	0.56	24.25
A0	81.8	1.91	1956.72
A2	2.8	2.23	34.83
E+			
D+			
S+			
C+			
	Total Area		2392.98

HbF 1.8 % **HbA2 2.8 %**

TP 964



[Unknown Peak]

Name	%	Time	Area
P00	1.7	0.29	39.67
P01	0.0	0.48	0.77
P02	0.1	0.65	3.02
P03	1.5	0.81	36.14
P04	5.1	1.20	121.61
P05	2.6	1.39	61.95
P06	1.9	1.49	45.66